

SM PHARMACEUTICALS SDN. BHD.

FUNGI-STAT CREAM

(ECONAZOLE NITRATE AND TRIAMCINOLONE ACETONIDE CREAM)

DESCRIPTION:

A smooth, soft, white, homogenous cream.

COMPOSITION:

Econazole Nitrate 10 mg / gram (1% w/w)

Triamcinolone Acetonide 1 mg / gram (0.1% w/w)

ACTIONS AND MODE OR MECHANISMS OF ACTION:

Econazole Nitrate

Fungistatic; may be fungicidal, depending on concentration; inhibits biosynthesis of ergosterol or other sterols, damaging the fungal cell wall membrane and altering its permeability; as a result, loss of essential intracellular elements may occur; also inhibits biosynthesis of triglycerides and phospholipids by fungi; in addition inhibits oxidative and peroxidative enzyme activity, resulting in intracellular buildup of toxic concentration of hydrogen peroxide which may contribute to deterioration of subcellular organelles and cellular necrosis. In *Candida albicans*, inhibits transformation of blastospores into invasive mycelial form.

Also have some activity against Gram-positive bacteria.

Triamconolone Acetonide

Triamcinolone diffuse across cell membranes and complex with specific cytoplasmic receptors. These complexes then enter the cell nucleuc, bind to DNA (chromatin), and stimulate transcription of messenger RNA (mRNA) and subsequent protein synthesis of various enzymes thought to be ultimately responsible for anti-inflammatory effects of topical application of adrenocorticoids.

PHARMACOLOGY (SUMMARY OF PHARMACODYNAMICS AND PHARMACOKINETICS):

Absorption: Minimal systemic absorption following topical application to normal skin.

Stratum corneum concentration

Far exceeded minimum inhibitory concentrations (MICs) for dermatophytes; inhibitory concentrations found in epidermis and as deep as middle region of dermis.

Elimination:

Renal and fecal; <1% of applied dose recovered in urine and feces.

Absorption: Dermal penetration, minimal systemic absorption.

TRIAMCINOLONE ACETONIDE

Absorbed systemically across the stratum corneum; occlusion, use over extensive areas, or prolonged use increases absorption of topical dosage forms.

INDICATIONS:

Eczema: unresponsive to less potent corticosteroids, inflammatory dermatoses with mycotic or bacterial infection. Inflammatory dermatomycoses caused by dermatophytes, yeasts and moulds. Mycotic infections in the body folds.

CONTRAINDICATIONS:

Patients hypersensitive to active ingredients or any other ingredients of the cream. Tubercular of the skin including vaccini, varicella and herpes simplex.

Patients with sensitivity to imidazoles have also reported sensitivity to econazole nitrate.

SIDE EFFECTS / ADVERSE REACTIONS:

Fungi-Stat should be well tolerated. Hypersensitivity (burning, itching, stinging, redness or other signs of irritation not present before therapy) was observed in very rare cases only. Systemic absorption of topical corticosteroids has produced reversible hypothalamic, pituitary-adrenal (HPA) axis suppression, manifestations of Cushing's syndrome, hyperglycemia and glucosuria in some patients.

The following local effects are reported infrequently with topical steroids but may occur more frequently with the use of occlusive dressings: Dryness, Folliculitis, Hypertrichosis, Striae, Hypopigmentation, Perioral dermatitis, Allergic contact dermatitis, Maceration of the skin, Secondary infection, Skin atrophy, Acneiform eruption

PRECAUTIONS / WARNINGS:

When this medication is used in the treatment of candidiasis, occlusive dressings should be avoided since they provide conditions, which favour growth of yeast and release of its irritating endotoxin.

If marked irritation or sensitivity occurs the treatment should be discontinued;

Avoid contact with eyes;

Compliance with full course of therapy; fungal infections may require prolonged therapy;

Use hygienic measures to help cure infection or to help prevent reinfection;

Check with physician if symptoms do not improve within 1 week or condition becomes worse.

Warnings:

Avoid prolonged application to thin skin and to the face. Long-term continuous topical therapy should be avoided where possible, particularly in infants and children, as adrenal suppression can occur even without occlusion. Prolonged use particularly with occlusive dressings may cause skin atrophy, telangiectasia and striae. The face, more than other areas of the body, may exhibit atrophic changes after prolonged treatment with potent topical corticosteroids. If used in childhood or on the face, courses should be limited if possible to five days and occlusion should not be used.

Paediatrics:

Children and adolescents have a large skin surface area to body weight ratio and less developed, thinner skin, which may result in absorption of greater amounts of topical corticosteroids. Absorption also is greater in premature infants than in full term newborns, due to inadequate development of the stratum corneum. It is recommended that only low-potency, unfluorinated topical corticosteroids that have a free 17-hydroxy group be used in children or growing adolescents.

Geriatrics:

Elderly patients may be more likely to have pre-existing skin atrophy secondary to aging. Purpura and skin lacerations that may raise the skin and subcutaneous tissue from deep fascia may be more likely to occur with the use of topical corticosteroids in geriatric patients. Therefore, topical corticosteroids should be used infrequently, for brief periods or under close medical supervision in patients with evidence of pre-existing skin atrophy. Use of lower potency topical corticosteroids also may be necessary in some patients.

Pregnancy:

Topical corticosteroids should not be used extensively, in large amounts or for prolonged periods in pregnant patients. Appropriate studies in humans have not been done.

Lactation:

It is not known whether econazole is distributed into human breast milk. However, problems in humans have not been documented. Safety of the use of topical adrenocorticoids during lactation has not been established. Topical corticosteroids should not be applied to the breast prior to nursing.

DRUG INTERACTIONS:

There are no known drug interactions or physical incompatibilities between Fungi-Stat Cream and other drugs.

RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION:

Adults

Fungi-Stat should be rubbed into the affected area morning and evening. The duration of treatment ranges from 8 to 14 days depending on the severity of the condition and individual response to the first week of treatment.

Paediatric use

Paediatric patients may demonstrate greater susceptibility to topical corticosteroid – induced HPA axis suppression and Cushing's syndrome than mature patients because of a larger skin surface area to body weight. Caution should be exercised.

SYMPTOMS AND TREATMENT FOR OVERDOSE AND ANTIDOTES:

Topically applied corticosteroids can be absorbed in sufficient amounts to produce systemic effects. Overdose with econazole nitrate has not been reported to date. In the event of accidental ingestion, nausea, vomiting and diarrhea may occur. If necessary, treat symptomatically.

DOSAGE FORMS AND STRENGTHS AVAILABLE:

Fungi-Stat Cream 1% w/w;

PACKING / PACK SIZES:

Aluminium collapsible tubes of 15 grams .

STORAGE CONDITIONS, USER INSTRUCTIONS AND PHARMACEUTICAL PRECAUTIONS:

Store below 30°C. Protect from freezing.

For external use only. Complete the treatment as prescribed by the physician.

SHELF LIFE: 3 years

MANUFACTURER AND PRODUCT REGISTRATION HOLDER:

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